

24SMCV05390 24SMCV05390

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Case Number: 24SMCV05390 Hearing Date: July 7, 2026 Dept: 207

TENTATIVE

RULING

DEPARTMENT

207

HEARING DATE

June

18, 2026 -- continued to July 7, 2026

CASE NUMBER

24SMCV05390

MOTION

Motion

for Summary Judgment

MOVING PARTIES

Defendants Cedars-Sinai Surgery Center of
the Pacific; Cedars-Sinai Health System; Cedars-Sinai Kerlan-Jobe Institute;
and Kevin Ehrhart, M.D.

OPPOSING PARTIES

Plaintiffs

Judith Seymour and Eugene Seymour

MOVING PAPERS:

Notice

of Motion and Motion for Summary Judgment; Memorandum of Points and Authorities

Separate

Statement of Undisputed Material Facts

Request

to Lodge Plaintiff's Original Deposition Transcript

Declaration

of Lorraine K. Hall

Declaration

of Donald B. Longjohn, M.D.

OPPOSITION PAPERS:

Opposition to Motion for Summary

Judgment; Memorandum of Points and Authorities

Declaration of Kenneth Yaeger

Declaration of Geni Bennetts, M.D.

Compendium of Exhibits

Declaration of Judith Seymour

Opposition to Separate Statement

Declaration of Jason Brustein,

D.O.

Objections to the Declaration of
Donald B Longjohn, M.D.

REPLY PAPERS:

Reply
in Support of Motion for Summary Judgment

Objections
to Evidence in Opposition to Motion for Summary Judgment

SUR-REPLY PAPERS:

1. Opposition to Motion for Summary
Judgment

2. Evidentiary Objections

BACKGROUND

This case arises from allegations that cement used in Plaintiff's knee replacement surgery became dislodged, causing her pain and disability.

On November 4, 2024, Plaintiffs Judith Seymour ("Judith") and Eugene Seymour ("Eugene") (together, "Plaintiffs") brought suit against Defendants Kevin M. Ehrhart, M.D.; Cedars-Sinai Kerlan-Jobe Institute; Cedars-Sinai Surgery Center of the Pacific; and Cedars-Sinai Health System ("Defendants"), alleging four causes of action for medical malpractice (against each named Defendant); (5) elder abuse; and (6) loss of consortium. The first five causes of action are alleged by Judith and the sixth cause of action for loss of consortium is alleged by her husband, Eugene.

On June 23, 2025, Plaintiff dismissed the fifth cause of action for elder abuse via oral motion.

Defendants now move for summary judgment on the grounds that Plaintiffs cannot establish breach or causation. Plaintiffs oppose the motion and Defendants reply.

LEGAL STANDARD – MOTION FOR SUMMARY

JUDGMENT

“[T]he party moving for summary judgment bears the burden of persuasion that there is no triable issue of material fact and that he is entitled to judgment as a matter of law[.]

There is a triable issue of material fact if, and only if, the evidence would allow a reasonable trier of fact to find the underlying fact in favor of the party opposing the motion in accordance with the applicable standard of proof.” (Aguilar v. Atlantic Richfield Co. (2001) 25 Cal.4th 826, 850.)

“[T]he party moving for summary judgment bears an initial burden of production to make a prima facie showing of the nonexistence of any triable issue of material fact; if he carries his burden of production, he causes a shift, and the opposing party is then subjected to a burden of production of his own to make a prima facie showing of the existence of a triable issue of material fact.” (Ibid.)

“On a summary judgment motion, the court must therefore consider what inferences favoring the opposing party a factfinder could reasonably draw from the evidence. While viewing the evidence in this manner, the court must bear in mind that its primary function is to identify issues rather than to determine issues. Only when the inferences are indisputable may the court decide the issues as a matter of law. If the evidence is in conflict, the factual issues must be resolved by trial.”

(Binder v. Aetna Life Ins. Co. (1999) 75 Cal.App.4th 832, 839

[cleaned up].) Further, “the trial court

may not weigh the evidence in the manner of a factfinder to determine whose version is more likely true. Nor may the trial court grant summary judgment based on the court’s evaluation of credibility.” (Id. at p. 840

[cleaned up]; see also Weiss v. People ex rel. Department of Transportation (2020) 9 Cal.5th 840, 864 [“Courts deciding motions for summary judgment or summary adjudication may not weigh the evidence but must instead view it in the light most favorable to the opposing party and draw all reasonable inferences in favor of that party”].)

EVIDENTIARY OBJECTIONS

The

Court rules as follows with respect to Plaintiffs’ evidentiary objections to the Declaration of Donald B. Longjohn, M.D.:

1. Overruled

2. Overruled

3. Overruled

4. Overruled

5. Overruled

6. Overruled

The Court rules as

follows with respect to Defendants' evidentiary objections:

1. Overruled

2. Sustained

3. Overruled

DISCUSSION

1. STATUTE OF LIMITATIONS

"In an action for injury [...]

against a health care provider based upon such person's alleged professional negligence, the time for the commencement of action shall be three years after the date of injury or one year after the plaintiff discovers, or through the use of reasonable diligence should have discovered, the injury, whichever occurs first." (Code Civ. Proc., § 340.5.)

Defendants argue Plaintiff's

claims are time barred because Judith admitted in deposition that she could have suspected wrongdoing right after her April 22, 2022 knee surgery, making the deadline to file April 22, 2023, yet Plaintiffs did not file the instant lawsuit until March 9, 2026.

As a threshold matter, the Court does not read the excerpts from Judith's deposition transcript the same way. Rather, Judith testified that while she was frustrated that she was being instructed to slow down her physical activities during the initial post-operative visits following the surgery, yet her swelling and pain persisted, she nonetheless "was relying on the expertise of my surgeon." (See Plaintiffs' Ex. at p. 36:1-19.) Plaintiff further testified she did not begin to become concerned until six months after the surgery, when she saw her own healing lagging behind friends who had undergone similar surgeries. (Id. at p. 42:25-43:17.)

Further, as Defendants point out in opposition, Dr. Ehrhart and Physician's Assistant Katherine Nelson Pavlik repeatedly reassured Plaintiffs during the post-operative appointments that her symptoms (and the retained cement) were either normal or Judith's own fault for overexerting herself.

The statute of limitations for professional negligence may be tolled beyond three years where there is intentional concealment or the presence of a foreign body, which has no therapeutic or diagnostic purpose or effect. (Code Civ. Proc., § 340.5.) Here, Dr. Ehrhart's and Ms. Pavlik's repeated post-operative assurances that there was no anomaly with regard to the surgery could be construed as intentional concealment, and the retained cement inside Judith's joint itself could be considered a foreign body, warranting tolling.

Therefore, the Court cannot find that the professional negligence claim is barred as a matter of law. There are triable issues of material fact underlying whether the statute of limitations is applicable.

2. PROFESSIONAL NEGLIGENCE

"Civil Code section 1714, subdivision (a) establishes the general duty of each person to exercise, in his or her activities, reasonable care for the safety of others. When applied to medical professionals, this duty of care imposes a duty to use such skill, prudence and diligence as other members of his profession commonly possess and exercise." (Flores v. Liu (2021) 60 Cal.App.5th 278, 290 [cleaned up].) To prevail on a claim for negligence against a medical professional, a plaintiff must demonstrate that: (1) a medical

professional had a duty to use the skill, prudence and diligence that members of the profession commonly possess and exercise; (2) breach of that duty; (3) an injury that resulted from the breach of that duty; and (4) actual loss or damage resulting from the breach of that duty.

(Banerian v. O'Malley (1974)
42 Cal.App.3d 604, 612.)

"[T]he legal standard of care required by doctors is the standard of practice required by their own profession. The courts require only that physicians and surgeons exercise in diagnosis and treatment that reasonable degree of skill, knowledge, and care ordinarily possessed and exercised by members of the medical profession under similar circumstances. Thus, liability is not found, and the label of malpractice is not placed upon a physician's actions, unless some deviation by the physician from the standard of care that his peers consider appropriate in the situation under review is proven." (Burgess v. Superior Court (1992) 2 Cal.4th 1064, 1081 [cleaned up].)

Expert testimony is generally the only admissible and relevant evidence on whether a medical professional has breached the standard of care. (Landeros v. Flood (1976) 17 Cal.3d 399, 410 [" 'The standard of care against which the acts of a physician are to be measured is a matter peculiarly within the knowledge of experts; it presents the basic issue in a malpractice action and can only be proved by their testimony (citations), unless the conduct required by the particular circumstances is within the common knowledge of the layman' "].)

As the Court of Appeal has held, in reversing summary judgments for medical professionals: "The standard of care against which the acts of a physician are to be measured is a matter peculiarly within the knowledge of experts; it presents the basic issue in a malpractice action and can only be proved by their testimony. California courts have incorporated the expert evidence requirement into their standard for summary judgment in medical malpractice cases. When a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, defendant is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence." (Hanson v. Grode (1999) 76 Cal.App.4th 601, 606-607 [cleaned up].) Further, a plaintiff "is entitled to all favorable inferences that may reasonably be derived from" an expert's declaration which must be liberally construed.

(See *Fernandez v. Alexander* (2019) 31 Cal.App.5th 770, 782.)

a. Moving Defendants' Evidence

In support of the motion,

Moving Defendants advance the expert declaration of Donald B. Longjohn, a board-certified orthopaedic surgeon who has been licensed to practice medicine in since 1996. Dr. Longjohn is also an Assistant Professor in the Department of Orthopaedic Surgery at the Keck School of Medicine, the Chief of the Surgical Arthritis Service and Rancho Los Amigos National Rehabilitation Center, and a Physician Specialist at Los Angeles General Medical Center. (Longjohn Decl. ¶ 2.) Dr. Longjohn is familiar with the standard of care for knee arthroplasty procedures, like that alleged by Plaintiff. (Longjohn Decl. ¶ 3.)

In forming his opinions, Dr.

Longjohn reviewed the complaint as well as Judith's medical records from Surgery Center of the Pacific, Cedars Sinai, UCLA Health Systems, Roaring Fork PT, Rite Aid, Providence St. John, Sports Fit & PT, Providence Medical Institute, Steadman Clinic, and Aspen Valley Hospital, as well as Plaintiffs' responses to Defendants' written discovery requests and the deposition transcripts for the depositions of both Plaintiffs. (Longjohn Decl. ¶ 4.)

In his review of the

materials, Dr. Longjohn's understanding of the basic facts are: after exhausting a series of injection treatments that provided only mild relief, Judith, who was then 76 years old, underwent a right knee medial unicompartmental arthroplasty with robotic assistance, performed by Dr. Ehrhart on April 22, 2022. (Longjohn Decl. ¶ 5 (a)-(i).) Following the procedure, Judith's pain level improved somewhat, but some pain persisted, especially as she continued working out every day, ultimately leaving Judith to feel disappointed with the procedure. (Longjohn Decl. ¶ 5(j)-(bb).)

A right knee arthroscopy with

synovectomy and loose body removal performed by Dr. Lee on February 9, 2024 revealed retained cement along the tibial tray and two fragments in the posterior joint measuring 10 x 4 x 8 mm and 12 x 6 x 2 mm. (Longjohn Decl. ¶ 5(qq).)

Based on his review of the

materials and understanding of the facts, Dr. Longjohn declared:

6. Based

upon my review of the materials in this case, and further based upon my education, training and experience, it is my opinion that the care provided to JUDITH SEYMOUR by Defendants were all within the standard of care.

7. As a

preliminary matter, CEDARS-SINAI KERLAN-JOBE INSTITUTE and CEDARSSINAI HEALTH SYSTEM their nurses, residents, and non-physician staff did not breach the standard of care. Plaintiff's allegations are related to an operation and post-operative care, including an alleged delay in diagnosis. These allegations are related to actions of a physician and not of CEDARS-SINAI KERLAN-JOBE INSTITUTE and CEDARS-SINAI HEALTH SYSTEM their nurses, residents, and non-physician staff.

8. There

was no breach in the standard of care by Defendant CEDARS-SINAI SURGERY CENTER OF THE PACIFIC. The only day that Plaintiff was seen at CEDARS-SINAI SURGERY CENTER OF THE PACIFIC was the date of the operation. There are no allegations that any of the nurses, residents or non-physician staff at CEDARS-SINAI SURGERY CENTER OF THE PACIFIC breach the standard of care. Rather, the allegations are related to the operation performed by Dr. Erhart. Dr. Erhart is the surgeon and the captain of the ship. There was no breach of the standard of care related to nurses, residents or non-physician staff at CEDARS-SINAI SURGERY CENTER OF THE PACIFIC because they did not perform the operation. Rather, Dr. Erhart performed the operation. As discussed below, there was no breach in the standard of care by Dr. Erhart related to the operation.

9. There

was no breach in the standard of care related to the decision of Dr. Ehrhart to perform the right knee medial unicompartmental arthroplasty with robotic assistance. The imaging on April 23, 2021, showed that the right knee medial joint space looked good, then on December 1, 2021, the joint space looked narrower, and finally then on March 1, 2022, the medial joint space had decreased further on right. These imaging studies coupled with the patient's report of discomfort make the operation indicated. Thus, there was no breach in the standard of care related to the decision of Dr. Ehrhart to perform the right knee medial unicompartmental arthroplasty with robotic assistance.

10. There

was no breach in the standard of care related to the performance of the right knee medial unicompartmental arthroplasty with robotic assistance by Dr. Ehrhart. Incidents of retained cement can be around 20% and can be retained absent negligence. The majority of these cases are asymptomatic with only a small number requiring surgery to remove cement fragments. With a partial knee, it is challenging to remove extruded cement because of the smaller incision which limits visibility and access to posterior extruded bone cement. There are tools to attempt to essentially blindly remove the cement, but it is challenging. In addition, there is a limited amount of time to do this since most of the extruded cement needs to be removed before it hardens, leaving only a few minutes to perform the task. Thus, with a partial knee there is limited visibility and limited physical access to the back of the knee creating in essence a blind spot behind the tibial implant, and as such, it is not uncommon for a breach in the standard of care for some cement to remain. Further, according to Dr. Lee, he had to break up much of the cement, which might mean that the cement was adjacent to the bone implant interface rather than free floating. Adherent extruded cement flush along the edge of the implant at the interface may be difficult to visually inspect and remove. Also, if this cement is fairly flush with the medial edge of the implant and bone, and is not loose or unstable, it may not cause any problems. Loose pieces of cement typically become a problem if they migrate into the joint causing impingement. Often these fragments remain in the posterior capsular recess and remain asymptomatic. Therefore, it is not a breach in the standard of care if there was cement retained.

11. As to

the follow-up care, it was reasonable and within the standard of care for Dr. Erhart and/or Katherine Nelson Pavlik, PA to believe that overuse was causing pain and swelling. Katherine Nelson Pavlik, PA is a physician's assistant and worked under the supervision of Dr. Erhart. The patient's pain would come and go. The records clearly reflect that she was very active and when she rested, she would improve. Finally, Katherine Nelson Pavlik, PA ordered imaging studies to determine if there was another source of the complaints. The results of the x-ray do not look consistent with how the symptoms presented. The little piece of cement on the x-ray was posterior but the pain she was reporting was more medial. On the x-ray, the little piece of cement was not big, but she was doing a lot. She was biking and running almost daily. In addition, she was walking the dog and working out. When she decreased activities, she was better. Thus, based on the clinical presentation, the patient's reports, and the imaging studies, it was reasonable and within the standard of care for Dr. Erhart

and/or Katherine Nelson Pavlik, PA to believe that the patient's symptoms were related to overuse. Therefore, there was no breach in the standard of care. In addition, there was a plan for an MRI if the symptoms did not improve. By the 3/1/2023 office visit the symptoms and appearance of the knee had improved so an MRI was not obtained. Dr. Ehrhardt did not discharge the patient from his care but asked her to followup with him in 3 months. The patient did not return to Dr. Erhart but instead chose to be evaluated at the Steadman Clinic.

12. It is

my opinion that, to a reasonable degree of medical probability, there was no act or omission on the part of the nurses, non-physician staff or residents at CEDARS-SINAI SURGERY CENTER OF THE PACIFIC, CEDARS-SINAI HEALTH SYSTEM, and CEDARS-SINAI KERLANJOBE INSTITUTE that caused or contributed to JUDITH SEYMOUR's alleged injuries. In sum, based upon my education, training and experience, as well as my review of the materials outlined above, there was nothing that the nurses, non-physician staff or residents at CEDARS-SINAI SURGERY CENTER OF THE PACIFIC, CEDARS-SINAI HEALTH SYSTEM, and CEDARS-SINAI KERLAN-JOBE INSTITUTE did, or failed to do, that caused JUDITH SEYMOUR's alleged injuries. The allegations in this matter are related to surgical and attending physician care and not that of the nurses, non-physician staff or residents.

13. It is

my opinion that, to a reasonable degree of medical probability, there was no act or omission on the part of Dr. Erhart and/or Katherine Nelson Pavlik, PA that caused or contributed to JUDITH SEYMOUR's alleged injuries. To a reasonable degree of medical probability, fragments of retained acrylic bone cement are usually do not cause symptoms. In sum, based upon my education, training and experience, as well as my review of the materials outlined above, there was nothing that Dr. Erhart and/or Katherine Nelson Pavlik, PA did, or failed to do, that caused JUDITH SEYMOUR's alleged injuries.

(Longjohn Decl. ¶¶ 6-13.)

Therefore, Defendants have met

their initial burdens of production and persuasion to demonstrate that (1) none of the named Defendants breached the applicable standard of care and (2) none of the named Defendants caused or contributed to Judith's alleged injuries.

b. Plaintiff's Evidence

Having met their initial burdens, the burden shifts to Plaintiffs to create a triable issue of material fact. Plaintiffs advance the expert Declaration of Jason Brustein, D.O., a board-certified orthopaedic surgeon, who completed his residency in Pittsburgh 2019, fellowship in New Jersey in 2020, and currently practices as an orthopaedic surgeon in McDonough, Georgia. (Ex. A to Brustein Decl.)

Dr. Brustein reviewed Plaintiff's medical records from Kerlan-Jobe and the surgery center, Steadman Clinic, Compass Peak Imaging, Dr. Jared T. Lee, as well as Plaintiffs' depositions and Dr. Longjohn's declaration.

Dr. Brustein points out that Dr. Ehrhart's surgical report from the April 22, 2022 surgery indicates, "[a]ll excess acrylic was removed" and that cement was visible on the x-rays taken on October 7, 2022, yet Dr. Ehrhart's progress notes do not mention the cement, that Dr. Ehrhart improperly treated Judith with steroid injections, and that after a follow-up examination on March 1, 2023, Dr. Ehrhart inaccurately diagnosed Plaintiff with "tertiary synovitis." (Brustein Decl. ¶¶ 8, 12-15, 20-21.) Dr. Brustein further opines:

28. There is no acceptable reason for leaving behind visible or retrievable cement fragments within the joint space. This is particularly true here where there were many pieces, some of which measured more than three quarters of an inch.

29. While minimal microscopic residue may occur, retained macroscopic cement fragments are not an acceptable outcome and fall below the standard of care.

30. In this case, retained cement fragments were left within the joint following surgery.

31. These fragments were not microscopic or trivial-they were sizable and visible on imaging.

32. The presence of such retained cement establishes that:

- The cement was not adequately visualized and removed intraoperatively; and/or

- The joint was not properly irrigated to remove residual cement debris

33.

Because the operative field is fully visible during cementing and cleanup, these fragments should have been identified and removed before closure. That is required by the standard of care.

34. The

failure to remove these cement fragments constitutes a clear breach of the standard of care.

35. In

addition to the intraoperative breach, Defendants failed to properly diagnose the cause of Plaintiffs postoperative symptoms.

36.

Postoperative x-rays demonstrated retained cement within the knee joint.

37. At

the same time, imaging showed:

-

Well-positioned prosthetic components

- No

fractures

- No

dislocations

- No

degenerative abnormalities

38. In

this clinical context, the most conspicuous and medically probable cause of

Plaintiff's ongoing pain and swelling was the retained cement. Plaintiff testified that Dr. Ehrhart acknowledged to her when he drained the knee that the blood fluid removal indicated that something was irritating the knee but that he did not see anything on the x-rays. Clearly the cement was on the x-rays and thus Dr. Ehrhart failed to properly diagnose the cause of Plaintiff's irritated knee despite having all the information he needed to do so.

39.

Despite this, Dr. Ehrhart repeatedly told Plaintiff that her symptoms were due to activity level or overuse.

40. This

conclusion fell below the standard of care because it ignored objective imaging findings and the known mechanical effects of intra-articular cement.

41. This

misdiagnosis delayed appropriate treatment and prolonged Plaintiff's suffering.

42. In

addition, the Cortisone injection is strongly discouraged in patient such as Plaintiff and not likely to result in benefit to the patient.

43.

Retained intra-articular cement is well known to cause:

- Mechanical irritation
- Synovitis and inflammation
- Persistent swelling
- Pain
with motion
- Joint
dysfunction

44.

Plaintiffs symptoms-particularly posterior medial and posterior lateral knee pain and swelling-are entirely consistent with mechanical irritation from retained cement.

45. No

amount of rest, activity modification, or conservative care could resolve symptoms caused by a retained foreign body within the joint.

46.

Plaintiffs continued pain persisted until the cement was surgically removed.

47. The

subsequent identification and removal of cement by Dr. Anderson and Dr. Lee confirm that the retained cement was the true source of Plaintiffs symptoms.

48. It is

my opinion, to a reasonable degree of medical probability, that:

- The

retained cement was a substantial factor in causing Plaintiffs pain and dysfunction; and

- The

delay in diagnosis prolonged her pain and suffering unnecessarily

49. As a

result of Defendants' breaches of the standard of care, Plaintiff suffered:

-

Prolonged pain and swelling

- Loss of

function

- Delay

in appropriate treatment

- Need

for additional diagnostic workup

-

Requirement of revision surgery

50. These

injuries were avoidable had the standard of care been met both during surgery and in postoperative evaluation.

(Brustein Decl. ¶¶ 28-50.)

Thus,

Plaintiffs have met their burden of production to create triable issues of material fact as to whether Defendants Kevin M. Ehrhart, M.D. and Cedars Sinai Surgery Center of the Pacific breached the applicable standard of care by leaving retained cement inside the surgery site and whether the acts/omissions of those Defendants caused or contributed to Judith's injuries.

As

for Defendants Cedars-Sinai Kerlan-Jobe Institute and Cedars-Sinai Health System, Plaintiffs explained at the hearing that their theory of liability against these Defendants is vicarious, on the ground that Plaintiff's post-surgical medical records, including the one where Physician's Assistant Katherine Nelson Pavlik told Plaintiffs that having retained cement in the joint is normal, all identify Cedars-Sinai Kerlan-Jobe (Plaintiffs' Exhibit 5), and Defendants' response to Form Interrogatory No. 3.6 demonstrates that Cedars-Sinai Kerlan-Jobe is a dba for Cedars-Sinai Health System (see Plaintiffs' Exhibit 4.)

Thus,

the Court denies summary adjudication as to all Defendants.

3. LOSS OF CONSORTIUM

Because Eugene's sixth cause

of action for loss of consortium is premised entirely upon Judith's knee surgery injury, it rises or falls with the others.

As such, the Court similarly denies summary adjudication of Eugene's sixth cause of action for loss of consortium as to all Defendants.

CONCLUSION AND ORDER

Finding Plaintiffs have met their burdens of production to create triable issues of material fact as to whether Dr. Ehrhart of Cedars-Sinai Surgery Center of the Pacific and/or Physician's Assistant Katherine Nelson Pavlik of Cedars-Sinai Kerlan-Jobe dba Cedars-Sinai Health System breached the applicable standard(s) of care or caused or contributed to Judith's injuries (and Eugene's resulting loss of consortium) by leaving retained cement inside the joint during surgery and representing to Plaintiffs after the surgery that the retained cement was normal, the Court denies Defendants' motion for summary judgment.

Counsel for Defendants shall provide notice of the Court's Order/ruling, and file the notice with a proof of service forthwith.

DATED: July 7, 2026 _____/s/ _____

Michael
E. Whitaker

Judge
of the Superior Court